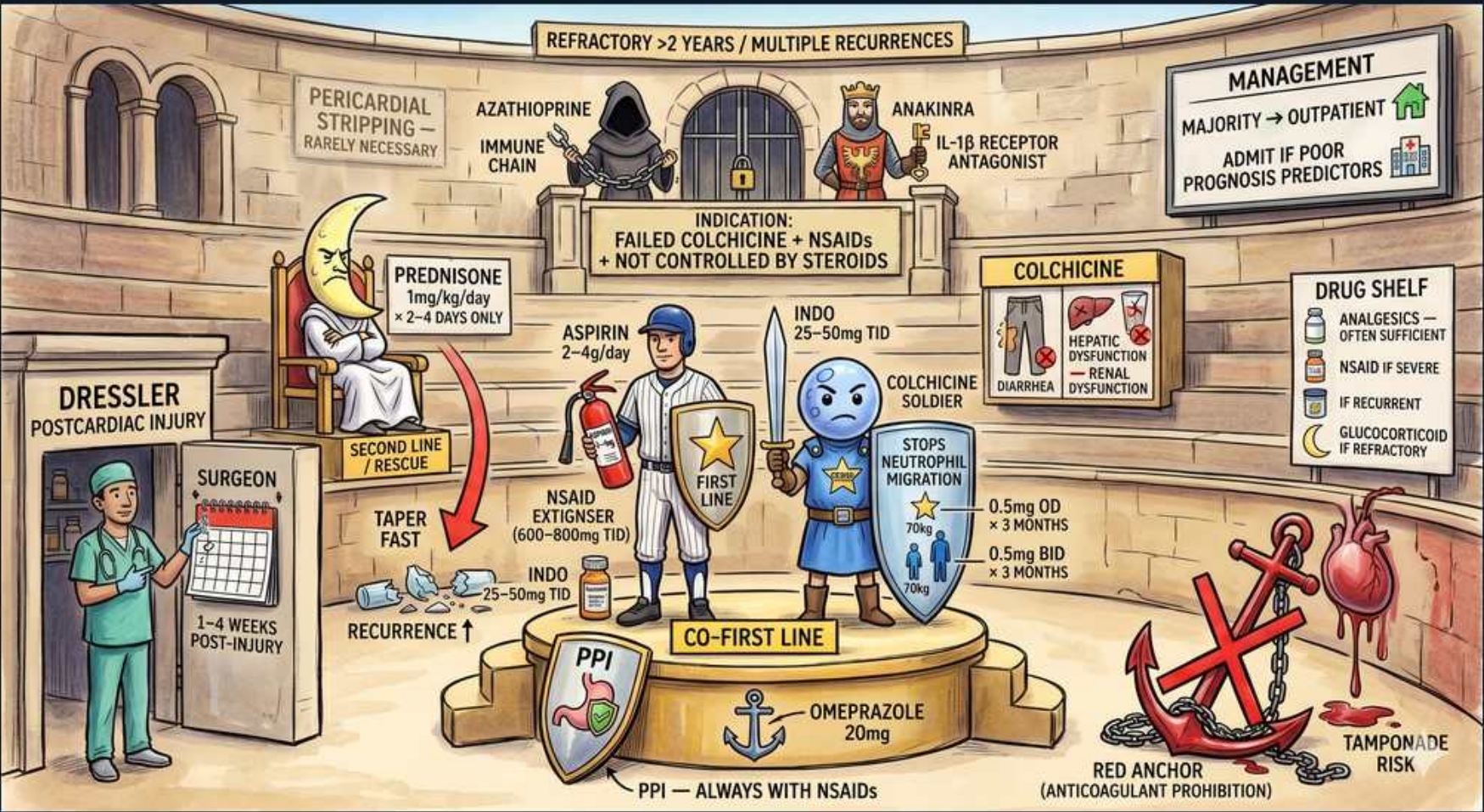


Pericardial Disease — Recurrent Pericarditis



1. Refractory definition

WHAT YOU SEE

Banner 'REFRACTORY >2 YEARS / MULTIPLE RECURRENCES'

WHAT IT ENCODES

Definitions to lock: RECURRENT pericarditis = a documented first episode, then a symptom-free interval of ≥ 4 –6 weeks, then relapse (occurs in ~15–30%, higher if colchicine was omitted). INCESSANT = symptoms persisting without a clear remission for 4–6 weeks to 3 months. CHRONIC = >3 months. REFRACTORY = multiple recurrences/steroid-dependence over years. Recurrence is diagnosed using the same criteria as acute pericarditis, with CRP useful to confirm and to guide tapering.

2. NSAID first-line

WHAT YOU SEE

NSAID extinguisher 'INDO 25–50mg TID', '600–800mg TID'

WHAT IT ENCODES

First-line is a high-dose NSAID/aspirin at full anti-inflammatory dose until symptoms resolve AND CRP normalizes, THEN taper — not a fixed short course. Options: ibuprofen 600–800 mg TID, indomethacin 25–50 mg TID, or aspirin 750–1000 mg TID (preferred post-MI). Always pair with a PPI and with colchicine.

3. Colchicine co-first-line

WHAT YOU SEE

'COLCHICINE SOLDIER' with shield 'STOPS NEUTROPHIL MIGRATION'

WHAT IT ENCODES

Colchicine is CO-FIRST-LINE and is the single most important drug for PREVENTING recurrence (inhibits microtubule polymerization → blocks neutrophil migration/inflammasome). Add it to NSAID/aspirin in BOTH the first episode and every recurrence. Course is 3 months for a first episode and ≥ 6 months for recurrent disease. It roughly halves recurrence rates (CORP/ICAP trials).

BOARD TRAP

Skipping colchicine raises recurrence risk. WRONG to treat acute or recurrent pericarditis with an NSAID/steroid alone. The discriminator: trials (ICAP, CORP, CORP-2) show colchicine added to NSAIDs cuts recurrence by ~50%, so omitting it is the classic management error that drives relapse and steroid dependence.

4. Colchicine dosing

WHAT YOU SEE

'0.5mg OD x3 mo' / '0.5mg BID x3 mo' by weight

WHAT IT ENCODES

Weight-based dosing: ≤ 70 kg → colchicine 0.5 mg once daily; > 70 kg → 0.5 mg twice daily. Duration 3 months for a first episode, ≥ 6 months (often longer) for recurrent disease. No loading dose is needed for pericarditis. Reduce dose with renal/hepatic impairment and avoid with strong CYP3A4/P-gp inhibitors (clarithromycin, etc.).

5. Colchicine side effects

WHAT YOU SEE

'HEPATIC/RENAL DYSFUNCTION', 'DIARRHEA'

WHAT IT ENCODES

Main adverse effects: GI (diarrhea, nausea — dose-limiting), and rarely myotoxicity/myelosuppression. Dose-reduce in hepatic or renal dysfunction and avoid co-administration with CYP3A4/P-glycoprotein inhibitors (macrolides, azoles, cyclosporine, statins) which raise colchicine levels toward toxicity.

BOARD TRAP

WRONG to continue full-dose colchicine with a strong CYP3A4/P-gp inhibitor (e.g., clarithromycin) or in severe renal/hepatic impairment. The discriminator: this drug interaction can precipitate life-threatening colchicine toxicity (severe diarrhea, myelosuppression, multi-organ failure) — adjust dose or hold.

6. PPI with NSAIDs

WHAT YOU SEE

'PPI — ALWAYS WITH NSAIDS', 'OMEPRAZOLE 20mg'

WHAT IT ENCODES

Co-prescribe a PPI (e.g., omeprazole 20 mg daily) for gastric protection whenever high-dose NSAIDs/aspirin are used, given the prolonged courses and high doses required in pericarditis. This reduces NSAID-induced ulcer/GI bleeding risk.

7. Steroids = 2nd line/rescue

WHAT YOU SEE

Prednisone king 'SECOND LINE / RESCUE', '1mg/kg x2–4 days'

WHAT IT ENCODES

Corticosteroids are SECOND-LINE/rescue only — reserved for NSAID+colchicine failure, contraindication/intolerance, or a specific indication (autoimmune disease, uremia, pregnancy). Use LOW-to-moderate dose prednisone 0.2–0.5 mg/kg/day (NOT high-dose 1 mg/kg, which worsens recurrence) and always continue colchicine alongside, then taper very slowly guided by CRP.

BOARD TRAP

Steroids are NOT first-line and increase recurrence if used early. WRONG to give prednisone up front for a typical viral/idiopathic episode. The discriminator: early/high-dose steroids promote chronic, steroid-dependent recurrent pericarditis; reserve low-dose steroids for refractory cases or specific etiologies, and keep colchicine on board.

8. Taper steroids slowly

WHAT YOU SEE

'TAPER FAST' (recurrence↑ if rapid) note

WHAT IT ENCODES

Taper corticosteroids SLOWLY and only after CRP has normalized; rapid tapering is a leading cause of relapse. Decrease in small increments over weeks-to-months, holding the dose if symptoms/CRP rebound. Maintain colchicine throughout the taper to cover the highest-risk window.

BOARD TRAP

WRONG to taper steroids quickly once the patient feels better. The discriminator: rapid steroid withdrawal precipitates recurrence; taper in small steps guided by CRP normalization and keep colchicine running to bridge the patient off steroids.

9. Indication for anakinra

WHAT YOU SEE

'INDICATION: FAILED COLCHICINE + NSAIDS + NOT CONTROLLED BY STEROIDS'

WHAT IT ENCODES

Escalate to IL-1 blockade (anakinra/rilonacept) for colchicine-resistant AND steroid-dependent recurrent pericarditis — i.e., the patient who relapses whenever steroids are reduced. IL-1 is the central cytokine driving the autoinflammatory recurrence loop, so blocking it is mechanistically targeted and steroid-sparing.

10. Anakinra (IL-1)

WHAT YOU SEE

King 'ANAKINRA — IL-1B RECEPTOR ANTAGONIST'

WHAT IT ENCODES

Anakinra is a recombinant IL-1 receptor antagonist; rilonacept (IL-1 α / β trap) is FDA-approved for recurrent pericarditis (RHAPSODY trial). Used for refractory, colchicine-resistant, steroid-dependent disease as a steroid-sparing agent. Main risk is injection-site reactions and infection; screen before starting.

BOARD TRAP

WRONG to reach for IL-1 blockers before adequately trying NSAID + colchicine and addressing reversible factors. The discriminator: anakinra/rilonacept are reserved for proven colchicine-resistant, steroid-dependent recurrence — not for a first recurrence that simply wasn't given colchicine long enough.

11. Azathioprine

WHAT YOU SEE

'AZATHIOPRINE — IMMUNE CHAIN'

WHAT IT ENCODES

Azathioprine is a steroid-sparing immunosuppressant for refractory recurrent pericarditis, useful to wean chronic steroids. It is slow in onset (weeks-months), so it is a maintenance/steroid-sparing agent rather than acute rescue. Check TPMT activity before starting and monitor CBC/LFTs.

12. Pericardiectomy last resort

WHAT YOU SEE

'PERICARDIAL STRIPPING — RARELY NECESSARY'

WHAT IT ENCODES

Pericardiectomy is a LAST resort for truly refractory recurrent pericarditis after maximal medical therapy (NSAID + colchicine + steroids + IL-1 blockade) has failed and quality of life is severely impaired. It is performed at experienced centers; it does not guarantee cure if visceral pericardium remains.

BOARD TRAP

WRONG to offer pericardiectomy early for recurrent pericarditis simply because episodes are frequent. The discriminator: surgery is reserved for genuinely refractory, medically-exhausted cases — most recurrent pericarditis is controlled with colchicine $\pm$ IL-1 blockade, and operating prematurely exposes the patient to unnecessary surgical risk.

13. Dressler / post-injury

WHAT YOU SEE

Surgeon 'DRESSLER POSTCARDIAC INJURY', '1–4 weeks post-injury'

WHAT IT ENCODES

Post-cardiac injury syndrome (Dressler after MI; also post-pericardiotomy/post-trauma) is an immune-mediated, often recurrent pericarditis arising ~1–4 weeks after myocardial/pericardial injury, with fever, pleuritis, and elevated ESR/CRP. Treat with aspirin (preferred post-MI) $\pm$ colchicine; colchicine also helps PREVENT post-pericardiotomy syndrome.

BOARD TRAP

WRONG to use non-aspirin NSAIDs or early steroids for Dressler/post-MI pericarditis. The discriminator: in the post-MI/post-injury setting choose ASPIRIN $\pm$ colchicine to avoid impairing myocardial healing; colchicine prophylaxis (COPPS trial) reduces post-pericardiotomy syndrome.

14. No anticoagulants

WHAT YOU SEE

Red anchor X 'ANTICOAGULANT PROHIBITION', 'TAMPONADE RISK'

WHAT IT ENCODES

Avoid/limit anticoagulants in acute or recurrent pericarditis with effusion because of the risk of hemorrhagic transformation of the effusion and tamponade. If anticoagulation is strictly required (e.g., mechanical valve), weigh risk carefully and monitor the effusion closely.

BOARD TRAP

WRONG to reflexively continue or start full anticoagulation in a patient with pericarditis and a pericardial effusion. The discriminator: anticoagulation can convert a benign effusion into a hemorrhagic one and precipitate tamponade — hold or minimize unless there is a compelling indication, and monitor with echo.